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Protocol for Clinical Guidance

Protocol for Development of American Association of Clinical Endocrinology Clinical Practice Guidelines and Consensus Statements: Summary of Methodology, Process, and Policy – 2023 Update

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ABSTRACT

Objective: This 2023 updated protocol summarizes the American Association of Clinical Endocrinology's (AACE's) new framework for the development of clinical practice guidelines and other guidance documents that includes changes to methodology, processes, and policies.
Methods: AACE has critically reviewed its development processes for guidance documents over the last several years against the National Academy of Medicine Standards for Developing Trustworthy Clinical Practice Guidelines and the Council of Medical Specialty Societies Principles for Development of Specialty Society Clinical Guidelines to determine areas for improvement.
Results: The new AACE framework for development of guidance documents incorporates many changes, including a revised conflicts of interest (COI) policy; strengthened commitment to collection of disclosures and management of relevant COI during development; open calls to membership for authors; new requirements for authors; new diversity, equity, and inclusion (DEI) policy; new empanelment process that incorporates consideration of DEI; and adoption of the Grading of Recommendations Assessment, Development and Evaluation (GRADE) methodology to increase the quality of evidence assessment and standardize recommendation grades and statements, among other improvements.
Conclusions: AACE has revised its policies and adopted a completely new methodology for guideline development in support of the mission to elevate the practice of clinical endocrinology to improve patient care. With the use of an evidence-based medicine framework and by continually assessing and improving its processes for development of guidance, AACE strives to deliver trustworthy, unbiased, and up-to-date information that ensures clinician and patient confidence in AACE content. Further, AACE hopes that these enhancements foster a more collaborative approach to development and increase engagement with the worldwide medical community to improve global health.
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Abbreviations

AACE, American Association of Clinical Endocrinology; CMSS, Council of Medical Specialty Societies; COI, conflicts of interest; CPG, clinical practice guideline; DEI, diversity, equity, and inclusion; GRADE, Grading of Recommendations Assessment, Development, and Evaluation; IOM, Institute of Medicine; NAM, National Academy of Medicine; PICO(TS), patient/population, intervention, comparator, outcome(s), time, and setting.

Introduction and Importance

Clinical practice guidelines (CPGs) are intended to assist health care professionals with medical decision-making to optimize patient care.[1] Development, implementation of, and adherence to CPGs that are trustworthy, evidence based, and collaborative have been shown to improve patient-centered outcomes in multiple medical settings.[2-4,5] Since its inception, the American Association of Clinical Endocrinology (AACE) has valued and prioritized the development of CPGs and consensus statements that provide clinical guidance for the endocrine community with a commitment to the integrity of its publications. AACE has critically reviewed its processes for guidance development over the last several years against the Institute of Medicine (IOM, now National Academy of Medicine [NAM]) standards and the Council of Medical Specialty Societies (CMSS) principles to determine areas for improvement.[1,6,7] This updated protocol summarizes changes to AACE processes, policies, and methodology for the development of AACE CPGs and other guidance documents. These changes support AACE's mission to elevate the practice of clinical endocrinology to improve patient care. With the use of an evidence-based medicine framework and by constantly assessing and improving its processes for development of guidance documents, AACE strives to:

- deliver trustworthy, unbiased, and up-to-date information,
- increase clinician and patient confidence in AACE content,
- demonstrate diversity and inclusion in both guidance and authorship,
- recognize health inequities,
- promote the use of AACE guidance,
- enhance relationships with other medical organizations to foster a more collaborative approach to development,
- and engage with the worldwide medical community to improve global health.

Development of AACE Guidance Documents: History

AACE, formerly known as the American Association of Clinical Endocrinologists, was founded in 1991 as an organization for endocrine-focused physicians with an emphasis on clinical knowledge and experience that supports the highest levels of patient care and standards of medical practice. Dedicated to its founding principles to prioritize patient-centered care and disseminate science-based education, AACE identified the need for evidence-based guidance and published its first CPG in 1995 in the inaugural issue of AACE's peer-reviewed scientific journal, *Endocrine Practice*, with a focus on clinical practice.[8] Since then, AACE has developed and published more than 100 guidance documents that have provided key recommendations for the care of patients with endocrine disorders. Many of these documents have become recognized landmarks in endocrine clinical practice. Topics addressed in these guidance documents have spanned the major endocrine disease states, including diabetes; nutrition and obesity; cardiometabolic syndrome and lipids; and thyroid, bone and parathyroid, pituitary, gonadal, adrenal, and neuroendocrine conditions.

In its first decade from 1991 to 2001, AACE established a publications committee with charges to manage the publication of *Endocrine Practice* and to institute policies and processes consistent with IOM standards and American Medical Association criteria before 2004 for the development of CPGs important to clinical endocrinology.[9] AACE recognized that the organization must develop trustworthy, evidence-based guidelines and adhere to a rigorous and transparent methodology with a systematic rubric for grading evidence. To meet this goal, an ad hoc task force of clinician members developed and published the first "Protocol for Standardized Production of Clinical Practice Guidelines" in 2004, which focused on the evidence-based medicine paradigm and included a template and checklist for the development of CPGs.[9] As experience and commitment increased, the guidelines program evolved to "build a pragmatic organizational infrastructure to improve white paper development," which encompassed CPGs and consensus statements, including algorithms.[10] The AACE protocol for the development of CPGs and other guidance documents was updated in 2010,[11] 2014,[12] 2017,[10] and now 2023 to refine the methodology and processes used.

The 2017 methodology introduced a newly created Scientific Referencing Team of clinicians, focused on updating ratings for evidence and grading of recommendations and ensuring a more transparent process. In 2018, AACE leadership created the CPG Oversight Committee, which has evolved to embrace its responsibility to oversee the critical priority of AACE guidance document development. Since its creation in 2018, the CPG Oversight Committee substantially revised policy and procedures for the development of AACE CPGs and consensus statements, updating or creating new processes for each stage. The CPG Oversight Committee is dedicated to effectively implementing AACE conflicts of interest (COI) and diversity, equity, and inclusion (DEI) policies related to empanelment and providing constructive internal reviews with the goal of producing evidence-based guidance documents of high quality that align with AACE positions.

In 2020, AACE embarked on an in-depth strategic evaluation of the organization to evolve its brand, policies, and procedures to better execute its mission of elevating the practice of clinical endocrinology to improve global health. Progression of the AACE brand included a name change—from endocrinologists to endocrinology—to reflect the inclusivity of its diverse membership who share collective clinical and scientific information, experience, and ideas and who collaborate to accelerate evidence-based practices. Since 2019, AACE has revised its COI policy several times—most recently in 2022—and committed to the careful consideration of disclosures for empanelment of authors on task forces as well as the management of relevant COI during the development of AACE guidance documents. In 2020, AACE established a DEI policy to lay the foundation for an organizational culture of respect, inclusion, belonging, and diversity and also created DEI committees for both members and staff to carry out the policy through meaningful change and opportunities. The AACE guidelines program immediately began implementing the new DEI policy in the selection of task force members and members of the CPG Oversight Committee, with consideration of a broad spectrum of diversity components.[13] The CPG Oversight Committee created empanelment workgroups that include a DEI clinician-representative to reduce bias in the

selection of authors. Finally, in 2020, a Guideline Strategic Planning Task Force met to identify challenges and needs of the AACE guidance document program and created a CPG Oversight Committee Workgroup to strategize solutions for process improvement in the development of AACE CPGs and consensus statements.

AACE's strategic plan between 2022 and 2025 supports the goals to increase awareness of AACE scientific publications and education to the worldwide endocrine community, the breadth of membership to include the entire endocrine community, the value of engagement in AACE, and the use of AACE education, guidelines, and publications. Toward these goals, in 2022, the AACE Board of Directors approved the adoption of a more rigorous methodology for the development of CPGs, an enhanced COI policy, increased DEI on task forces, and more robust dissemination of AACE CPGs and consensus statements, including algorithms.

AACE Guidance Documents

AACE develops multiple types of guidance documents to assist members and their care teams in providing patient care of the highest quality. As summarized in Table 1, the evidence-based methodology for development of CPGs is more formal and systematic than for consensus statements. However, all AACE guidance documents undergo standardized and objective empanelment according to policies and procedures, review by the CPG Oversight Committee and the AACE Board of Directors, and peer review by *Endocrine Practice* before acceptance.

Clinical Practice Guidelines

Standards and Principles

CPGs include evidence-based recommendations to improve patient care and represent the mission and position of AACE on specific clinical endocrinology topics. CPGs follow a rigorous and time-intensive process (Fig.) based on the IOM/NAM standards and this most recent AACE protocol for guideline development. The IOM/NAM produced 2 seminal resource books on developing trustworthy guidelines in 2011, which are still followed worldwide today: *Standards for Developing Trustworthy Clinical Practice Guidelines* and *Finding What Works in Health Care: Standards for Systematic Reviews*.[1,7] The NAM states that "standards for developing clinical practice guidelines and recommendations… should ensure that clinical practice guidelines are unbiased, scientifically valid, and trustworthy and also incorporate separate grading systems for characterizing quality of available evidence and strength of clinical recommendations."[1] *Standards for Developing Trustworthy Clinical Practice Guidelines* covers topics such as management of COI, assessment of quality of evidence, transparency, rating strength of guideline recommendations, external review, and updating, among other topics.[1] *Finding What Works in Health Care* discusses additional challenges and provides guidance on the importance of multidisciplinary stakeholder input, the management of bias, and the need for rigorous systematic reviews of evidence.[7] The CMSS Principles for Development of Specialty Society Clinical Guidelines is another trusted resource used by AACE, which is based on the IOM/NAM standards and summarizes the most important steps "as a roadmap or set of aspirations" for its members to develop reliable evidence-based CPGs.[6]

Methodology

In May 2022, the AACE Board of Directors approved adoption of the Grading of Recommendations Assessment, Development and Evaluation (GRADE) method for the development of AACE CPGs. GRADE provides a transparent and systematic framework for development of evidence-based recommendations.[14] Since GRADE employs a standardized process and is used by more than 100 organizations worldwide, AACE recommendations will be consistent with those of many other collaborating organizations regarding assessment, grading, and generalizability to patients and health care professionals. Harmonization of recommendations across disciplines has importance for reducing conflicting guidance, thereby enhancing patient care, and is strongly encouraged by the IOM/NAM standards and CMSS principles. As part of its commitment to adoption of GRADE and elevating its guideline program, AACE joined both the GRADE Working Group and Guidelines International Network in 2022 to be part of global communities that collaborate on best practices in guideline development. The adoption of GRADE as the key methodology standard for AACE guidelines will ensure their trustworthiness and facilitate implementation.

The guideline development process begins with the establishment of key research questions in the PICO(TS) format: to identify the patient/population, intervention, comparator, outcome(s), and (optional) time and setting. Deliberate use of the PICO(TS) format informs the guideline's search parameters and inclusion/exclusion criteria. Search strategies are documented to increase transparency and reproducibility. Moving forward, AACE methodology staff will work with a medical librarian to design and conduct systematic literature searches. Evidence is selected according to inclusion and exclusion criteria determined a priori, and individual studies are assessed for risk of bias by the AACE methodology staff and a methodology fellow

Table 1
Comparison of AACE Clinical Practice Guidelines and Consensus Statements

Clinical Practice Guidelines	Consensus Statements
➢ Follow National Academy of Medicine (formerly Institute of Medicine) standards and the most recent AACE protocol for guideline development ➢ Address specific clinical questions based on PICO(TS) (population, intervention, comparator, outcome, time, setting) format ➢ Provide evidence-based recommendations to improve patient care ➢ Informed by systematic reviews and unbiased assessment of evidence, benefits, and harms, using the GRADE approach	➢ Follow a consistent yet less formal development process than CPGs ➢ Provide evidence-informed guidance in narrative and/or visual formats, including algorithms ➢ Referenced and scholarly, but do not require systematic appraisal of evidence

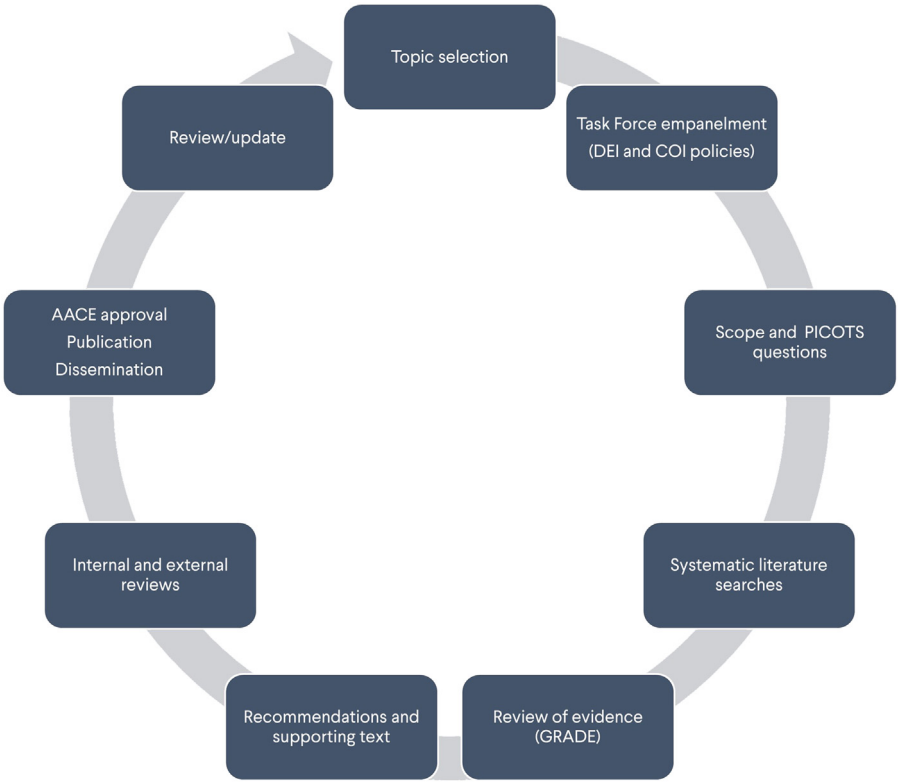

Fig. Life cycle of an AACE clinical practice guideline.

(see section on Training for GRADE below). AACE strives to limit the number of questions and scope of guidelines to facilitate implementation and adoption of guideline recommendations in clinical practice.

After completion of systematic reviews, CPG task forces use the GRADE tool to assess the strength of the evidence ("quality") for each outcome that is important or critical for clinical decision-making.[15] GRADE allows for upgrading and downgrading of the evidence, based on specific domains and study type.[16] Randomized controlled trials start out with a high certainty of evidence that can be downgraded based on risk of bias, imprecision around the estimate of effect, indirectness, inconsistency, and publication bias.[16] Nonrandomized studies start out with a low certainty of evidence and can be upgraded if there is a large effect size, a dose-response effect, or if an effect is observed despite the presence of potential confounding factors that would diminish that effect.[16] Once all domains for grading the evidence are assessed, the final GRADE certainty of evidence ranges from high when the task force is very confident in the estimate of effect to very low when the task force has little confidence in the estimate of effect (Table 2).[16] Table 2 shows the recommendation grading system adopted by AACE for CPGs. AACE issues strong recommendations based on moderate or high certainty of evidence and conditional recommendations based on lower certainty of evidence when treatment choices may vary based on patient preferences and values. AACE strives to provide transparent recommendations based on available evidence. For recommendations based on low certainty of evidence, frequent updates may be possible, as the potential is likely for additional studies to change the strength of the evidence.

GRADE evidence tables inform completion of GRADE Evidence-to-Decision Frameworks that provide a transparent mechanism to capture values and judgments of the task force toward development of recommendations. As shown in Table 3, the GRADE Evidence-to-Decision Framework uses a list of domains that enable a task force to make judgments about the balance of the desirable and undesirable effects (ie, benefits and harms) of an intervention, certainty of evidence, patient values related to selected outcomes, resources required, associated costs, and impacts on equity, acceptability, and feasibility of an intervention.[16-18] Based on these judgments and certainty of evidence, a task force determines the strength and direction of recommendations. Depending on the topic, as part of this new methodology, AACE CPGs may include some "good practice statements," which are based on expert opinion when well-designed supporting evidence is not available; but even in those cases, GRADE methodology will be followed, and the use of good practice statements should be limited.[19]

Training for GRADE

Implementation of GRADE as a new methodology has several layers. The first and most important step is to increase understanding of the GRADE method for CPG development in the AACE community. Staff worked with experts in GRADE methodology to develop recorded learning modules as a workshop specifically for AACE. All members of CPG task forces and the CPG Oversight Committee are required to complete these training modules prior to beginning work on a CPG. The guideline development modules, along with external training and support from staff methodologists, provide a foundation for AACE task force members to become more familiar with GRADE as they use this tool. In addition, AACE has created a methodology fellow program for any members to gain hands-on training in the development of CPGs and consensus statements. The goal of this burgeoning program is to enhance participants' knowledge and expertise in the methodology and development of evidence-based guidance documents. Looking to the future, training for the use of GRADE and development of evidence-based AACE guidance will not be limited to online modules

Table 2
American Association of Clinical Endocrinology Recommendation Grading System[a]

Recommendation Type	Definition	Certainty of the Evidence	
Strong AACE recommends for/against…	High confidence in the estimate of effect across outcomes. Most informed patients would choose the recommended option.	High Moderate	⊕⊕⊕⊕ ⊕⊕⊕○
Conditional AACE suggests…	Lower confidence in the estimate of effect across outcomes. Patient choices may vary based on values and preferences.	Moderate Low	⊕⊕⊕○ ⊕⊕○○
Good Practice Statement[b]	Ungraded guidance statements with no direct supporting evidence	none	

[a] Adapted from GRADE system for assessing the certainty of evidence and developing evidence-based recommendations.[15-17]
[b] Consistent with the GRADE methodology, good practice statements will be clear and actionable. Task forces will label good practice statements as ungraded, transparently report judgments which consider benefits and harms, and clearly document rationale of statement to indirect evidence.[18]

but will be dynamic and include in-person and video-based virtual training that will be offered to members of task forces as well as the CPG Oversight Committee. Beginning in 2022, a methodology fellow was selected for each task force, with the potential to increase the number of participants in future years. Members at any career level may apply for the position of methodology fellow during specified calls for authors.

Guideline Updates

Consistent with the IOM/NAM standards and CMSS principles, AACE makes every attempt to maintain the currency of its CPGs. AACE reviews and updates or retires its evidence-based guidelines every 3 to 5 years or after significant scientific developments or changes in public policy. Approval of updates by the CPG Oversight Committee and AACE leadership depends on the topic, scientific advances, new evidence since last publication that impact recommendations, organizational priorities, and available resources. Subject matter expert members of the AACE Disease State Network branch of the organization support formal review of CPGs and provide recommendations to the CPG Oversight Committee regarding update or retirement. A guideline is considered for update based on its relevance to AACE members, potential to address gaps in practice, and potential to improve patient care. All new or updated CPGs require de novo development until all guideline recommendations adhere to the same GRADE approach and new processes. Finally, for transparency and accordance with the IOM/NAM standards and CMSS principles, the updating process will be described in the methods section of each CPG.

Consensus Statements

As outlined in Table 1, AACE consensus statements provide evidence-informed guidance that reflects the opinion of subject matter experts and represent the mission and position of AACE on specific topics in clinical endocrinology. In contrast to CPGs, AACE consensus statements do not require a systematic appraisal of evidence and follow a less formal development process. However, AACE consensus statements, including algorithms, provide guidance in a narrative and/or visual format that is well referenced following a review of available literature by task force members.

It is important to clarify that consensus statements are not the same as narrative/literature reviews of a topic. Similar to CPGs, consensus statements are official AACE guidance documents that are empaneled according to COI and DEI policies and undergo formal review by the CPG Oversight Committee and AACE Board of Directors. To support AACE's commitment to DEI, consensus statements also consider health disparities/equity.

Ensuring Trustworthiness of AACE Guidance Documents

Trust is foundational to the art of medicine and the clinician-patient relationship and therefore is essential in the development of CPGs and other guidance documents. To increase the trust of clinicians, patients, and the public, AACE strives to create CPGs and consensus statements with minimal bias, meaning that task forces will be multidisciplinary, have limited COI, and use a transparent framework for the inclusion and evaluation of evidence when developing clinical recommendations and practice points.

As outlined in the IOM/NAM Standards and the CMSS Principles, AACE CPGs should clearly outline methods for systematic reviews of literature so that the reviews can be reproduced by others. In addition, evaluation of evidence will be conducted using a method that is understandable and transparent with strengths and limitations adequately discussed, resulting in recommendations firmly rooted in evidence. The strength of a recommendation will correspond with the certainty of evidence with considerations of benefits, harms, and patient preferences and values. In this way, the end user of AACE CPGs can effectively apply recommendations in their specific clinical setting and in the context of shared decision-making.

Management of Conflicts of Interest

To be consistent with the IOM/NAM standards and CMSS principles, AACE over time has applied an increasingly rigorous and

Table 3
GRADE Evidence-to-Decision Framework Domains[a]

Problem	Is the problem a priority?
Desirable effects	How substantial are the desirable effects (across outcomes)?
Undesirable effects	How substantial are the undesirable effects?
Certainty of evidence	What is the overall certainty of evidence?
Values	Is there important variability in how people would value the outcomes?
Balance of effects	Does the balance of effects favor the intervention or the comparator?
Resources required	Are there substantial costs or resources required for the intervention?
Cost-effectiveness	What is the cost-effectiveness of the intervention?
Equity	What are the effects on equity?
Acceptability	Is the intervention acceptable to stakeholders?
Feasibility	Is the intervention feasible for stakeholders to implement?

[a] Domains from the GRADE Evidence-to-Decision Framework adapted from Alonso-Coello et al.[16]

transparent process for limiting potential bias in development of guidance documents. To this end, AACE updated its process for reporting and review of relevant financial and intellectual disclosures of interest and its policy for determination and management of COI in 2022 and requires disclosures from all individuals involved in all stages of development for AACE CPGs and other guidance documents.[20]

Task force applicants are required to disclose all relationships and interests during the preceding 24 months, including but not limited to stock, company, or product ownership; consulting; research; speakers' bureaus; and activities or leadership positions at other organizations. The COI Subcommittee under the purview of the CPG Oversight Committee reviews disclosures in accordance with AACE policies and determines COI that are relevant to the guidance document topic. Applicants are not eligible to serve on an AACE task force if their potential COI cannot be managed effectively.[20] AACE requires the majority of task force members (60%), including chairs/co-chairs/vice chairs, to be free of COI related to the topic under development. The remaining 40% of task force members may be appointed with COI if those conflicts are determined to be manageable based on the type and extent of the relationship.[20] However, AACE strives to form task forces and workgroups with minimal COI. Further, AACE requires that chairs of task forces are free of relevant COI for the 24 months prior to appointment and remain free of relevant COI during development of the document and for 12 months after publication. In rare instances when a chair with specific expertise and relevant COI is deemed necessary for development of a guidance document, a co-chair with no COI must be appointed.[6,20]

AACE is committed to careful management and transparency of disclosures and COI for all who participate in the development of AACE guidance documents. To support and simplify this process, AACE maintains a central electronic repository for disclosures. To facilitate review of disclosures and management of COI, subject matter experts in AACE Disease State Networks assist the CPG Oversight Committee in creating lists of products, technologies, and companies specific to a guidance document topic for use by the COI Subcommittee, CPG Oversight Committee, chairs of task forces, and staff. Chairs and/or staff share new or updated disclosures and COI of all task force participants, as well as AACE committee members, at the beginning of every meeting. Management of conflicts depends on the nature of the relationship and may include verbal reminders to the group prior to discussion and voting, recusal from discussion, and recusal from voting on recommendations related to the COI, which will be documented. Every AACE guidance publication includes disclosures of all authors and a summary of the AACE COI Policy.

Member Review and Comment

Beginning in 2023, AACE members may review and comment on AACE CPGs during a specified period, in addition to review by the CPG Oversight Committee, AACE Board of Directors, and co-sponsoring organizations, if applicable. Based on appraisal of member comments, the CPG Oversight Committee will recommend to task force chairs any appropriate or necessary revisions. Expansion of the review process to include the AACE membership boosts participation and inclusivity of members in the organization and helps to ensure that AACE CPGs are valuable, useful, and actionable by clinicians and their care teams. Final organizational review of guidance documents is conducted by the CPG Oversight Committee that makes recommendations to the AACE board for approval. With board approval, AACE guidance documents are submitted to the journal *Endocrine Practice* for peer review prior to acceptance.

Collaboration with Other Medical Organizations

Following the IOM/NAM standards and CMSS principles, AACE champions collaboration in the development of CPGs to:

- Harmonize recommendations among relevant organizations and reduce conflicting or duplicative guidance
- Increase credibility, acceptance, and adoption of recommendations across multiple specialties
- Maximize and share resources to provide clinical practice guidance for AACE members, their care teams, and patients, which, given the resource-intensive nature of development, may not have been feasible otherwise

AACE principally collaborates in the development of CPGs and other guidance documents either through co-sponsorship or endorsement. Each type of collaboration follows a set process that includes peer review of the draft document by the appropriate Disease State Network members and the CPG Oversight Committee, and review and approval by AACE leadership.

Co-sponsorships initiated by AACE or from external organizations are approved by AACE leadership and include a formal agreement outlining the roles and responsibilities of the co-sponsoring organizations, including development and approval processes, peer review, publication, and dissemination of the document. AACE representative(s) on guidance documents developed by external organizations are nominated by the CPG Oversight Committee in accordance with COI and DEI policies.[13,20] Likewise, for AACE-initiated co-sponsorships, representatives from the co-sponsoring organization(s) are included on task forces per AACE COI and DEI policies. Through joint development, AACE provides a broader platform for dissemination and educational opportunities for up-to-date and evidence-based guidance to enhance care of patients with endocrine disorders.

Increasing Diversity, Equity, and Inclusion

AACE is committed to diversity, inclusion, equal representation, and access for all and strives to ensure DEI across the organization. AACE intends to fully represent its constituency and to foster DEI in the AACE community, including its membership, leadership, governance, publications, and educational programs. To enhance DEI, AACE encourages participation of the full membership in development of guidance documents. For recruitment and selection of authors, task force empanelment includes an open call to AACE membership to solicit volunteers to serve as authors as well as a formal review of applicants with respect to DEI. Prospective authors must complete an application that collects information on geographic location, work setting, career level, and a statement about DEI. The first calls for authors were initiated in the summer of 2022 for several guidance documents that are in process. Additional calls for authors will be disseminated to AACE members through multiple communication channels including email, AACE.com, and social media with the opportunity to apply for chair, vice chair, author, and methodology fellow of a task force.

As part of a new AACE process for development of guidance documents, empanelment workgroups include representatives from the CPG Oversight Committee, Disease State Network relevant to the topic, and DEI member committee. The assigned empanelment workgroup carefully reviews applications for experience, expertise, demographic information, and support for DEI, in addition to applicants' curricula vitae and other information, when selecting potential candidates for service. The COI Subcommittee reviews disclosures and determines any relevant COI of applicants. Empanelment workgroups are encouraged to rate all candidates

impartially, to avoid selecting a majority of candidates from the same institution/region/background/gender or similar to those selecting, and to consider objective skills and abilities over subjective opinions. All members of the empanelment committee receive training on decreasing bias in the selection of applicants.

Incorporation of DEI in empanelment results in task forces that reflect the AACE community with increased participation across demographics and professions (eg, MD, PhD, PharmD, RN, NP). Such egalitarian task force empanelment enriches the organization's culture that ensures equitable access to resources and opportunities, engages multiple perspectives, and encourages contributions of the broader membership. This multidisciplinary representation provides a comprehensive base of knowledge, experience, and varied perspectives to inform, strengthen the generalizability of, and reduce bias in AACE evidence-based guidance. These new processes align with several respected guideline development checklists and appraisal tools such as the Guidelines International Network/McMaster Guideline Development Checklist and the Appraisal of Guidelines, Research, and Evaluation (AGREE)-II Tool.[21,22]

Not only is the selection of task force members who develop guidance documents conducted resolutely with DEI as a frame of reference, but also the CPG Oversight Committee itself was restructured with considerations of DEI. The Oversight Committee has expanded recruitment to include subject matter experts outside endocrinology (eg, primary care) and added an endocrinology fellow-in-training to engage member-volunteers at earlier career stages and to create pathways for leadership in the association. As the CPG Oversight Committee governs creation and prioritization of science-based publications, the purpose of restructuring this committee is to increase diversity and inclusivity of thought that is critically important for the final review of AACE guidance documents.

In conjunction with DEI efforts at the organizational level, AACE is also committed to addressing health equity and social determinants of health in all guidance documents. Task forces are expected to consider impacts on health equity in all recommendations, include data/evidence on the role of social determinants in patient outcomes, provide information or resources to aid in addressing these issues as part of any treatment or management plans, and use person-first and inclusive language in guidance documents and supporting educational activities. In addition, cultural competency and transcultural differences should be considered in recommendations and in implementation of the guidance document.[23] Practical, actionable, DEI-oriented recommendations for clinical practice advance AACE's commitment to improving health for all patients.

Current Progress

AACE fully implemented its updated COI and DEI policies and procedures for the development of guidance documents in 2022. In conjunction with other process changes outlined above, these actions have increased the diversity and inclusion of members serving on guidance document task forces and assured that equity is part of the process. These principles continue to strengthen the integrity and minimize the bias of AACE publications. In addition, implementation of GRADE methodology is underway. The first guideline using GRADE is targeted for completion in 2024.

A comprehensive marketing plan for AACE CPGs was launched in 2022 to increase public awareness, dissemination, and impact of AACE guidance. AACE is working to enhance engagement with current collaborators while exploring new partnerships with other medical organizations to aid in harmonizing clinical guidance and reducing duplicative efforts to advance the delivery of high-quality patient care and improve global health.

Future Directions

Transformative steps have occurred to advance the AACE guidance document program over the last several years to standardize processes and refine or create new policies to reflect best practices, but additional changes are still intended. AACE aspires to update its guidelines in a timely manner and provide clinical guidance in a clear and user-friendly format. The CPG Oversight Committee hopes to continue broadening recruitment of members who are trained in evidence-based medicine in general and the GRADE method in particular through workshops, participation of experienced task force members, and expansion of the methodology fellows program. In addition, AACE has embraced not just virtual but in-person training for members of task forces and the CPG Oversight Committee with the purpose of creating committees that have the evidence-based medicine tools to produce guidance products and education at the highest scientific level.

Longer-term goals include establishing more defined target measures for DEI and involving the participation of patient/public stakeholders as well as nonphysician care team members in the development of AACE guidance documents. Inclusion of key stakeholders is essential to the successful consideration of patient preferences and values in recommendations for clinical care, particularly those in which the certainty of evidence is not high. Inclusion of stakeholders can take various forms, through use of public comment, inclusion as members on task forces, or using surveys, patient advisory panels, and studies available in the literature. AACE plans to use all options as determined by best practices and as appropriate for the topic of the guidance document.

Conclusions

The mission of AACE is to elevate the practice of clinical endocrinology to improve global health. AACE's core values of patient-centered care, equity, inclusivity, collaboration, excellence, and innovation are central to its guidance document program with the goal to be a trusted source of information and to achieve the vision of healthier communities through endocrine education and care.

Disclosures

R.C. is the current chair of the AACE CPG Oversight Committee, a member of the AACE Board of Directors, and in leadership positions at the American Medical Association, Western Endocrine Alliance, American Federation of Medical Research, Association of Program Directors in Endocrinology, Diabetes and Metabolism, and Endocrine Society. J.M. is the current vice-chair of the AACE CPG Oversight Committee and in a leadership position at the Georgia Society of Endocrinology. T.H. is on the advisory board (no financial compensation) for Horizon therapeutics and a consultant for Acella (no financial compensation). S.J. is editor-in-chief of *AACE Clinical Case Reports*. D.C.L. is in leadership positions at the American Diabetes Association, Association of Program Directors in Endocrinology, Diabetes and Metabolism, and Endocrine Society. S.L.W. is a past president of AACE and former chair of the AACE CPG Oversight Committee, and receives research support and is a national or overall principal investigator for Novo Nordisk. M.B., D.C., and C.S. have nothing to disclose.

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Author Contributions

R.C. and J.M. are co-first authors. M.D.B. and C.S. are co-senior authors.

Review Process

Drafts of this protocol update were reviewed and approved by all writing group members, the AACE CPG Oversight Committee, the AACE Board of Directors, and peer reviewers for *Endocrine Practice*.

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